

CHILD HEALTH & NUTRITION IN INDIAN MAINSTREAM MEDIA (Nov 2024 – May 2025)

Media Analysis Report for CRY Internship – Group B (Saanvi Nanda & Raashi Bhatjiwale)

1. Executive Summary

This report critically analyses how Indian mainstream media has covered issues related to child health and nutrition from November 2024 to May 2025. The analysis is based on over 25 articles sourced from English and Hindi language media, including Times of India, India Today, Navbharat Times, and Jansatta. The findings reveal a wide spectrum of themes: malnutrition, obesity, food safety, early childhood nutrition, dietary policies in schools, and WASH (Water, Sanitation and Hygiene). The study highlights the media's dominant focus on problem-based reporting, with limited but growing solution-oriented coverage. Government policies, expert commentary, and school-level interventions form the backbone of most stories; however, the direct voices of children remain largely absent. The report concludes with recommendations for more intersectional, child-centered, and data-rich narratives in the Indian media landscape.

- According to **NFHS-5**, 35.5% of children under five are stunted, 19.3% are wasted, and 32.1% are underweight.
 - India ranked **111 out of 125** in the 2023 Global Hunger Index.
 - Only **11.3% of children aged 6–23 months** receive an adequate diet (UNICEF India).
 - **Media plays a dual role**—agenda-setting and framing—that can either spotlight or suppress children's health concerns.
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2. Introduction

The portrayal of child health and nutrition in Indian media is not merely a reflection of public health realities—it also shapes societal attitudes and policy directions. This microstudy, conducted as part of the CRY internship program, aims to evaluate how mainstream media addresses children's nutritional needs, highlights or overlooks structural challenges, and whether it engages with children as active stakeholders or passive subjects. It is vital to assess if the media reinforces stereotypes or brings forward solution-driven discourse.

3. Methodology

Articles were gathered from English and Hindi publications using manual searches and media tracking tools. Each entry was analyzed based on:

- **Theme** (nutrition, malnutrition, food safety, obesity, etc.)
- **Tone** (objective, subjective, editorialized)
- **Treatment** (problem vs. solution focus, expert-driven, data-driven)
- **Authenticity** (sources cited, government or expert quotes)
- **Geographic focus** (national, regional, local)

The articles were tagged, and a comparative thematic analysis was conducted to identify key trends and patterns in media framing.

- A total of **28 articles** across 10 major publications (6 English, 4 Hindi) were reviewed.
 - Hindi publications included more **family-oriented or doctor-advice formats**, often informal but accessible.
 - Limitations included a lack of multimedia formats (videos, data visualizations), and zero inclusion of child testimonies.
 - No coverage in **regional vernaculars beyond Hindi** (e.g., Marathi, Tamil), missing key geographies.
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4. Key Findings

A. Dominant Themes

- **Malnutrition & Undernutrition:** Articles from Navbharat Times and Drishti IAS emphasized India's struggles with undernutrition, citing NFSA and POSHAN initiatives.
- **Obesity & Lifestyle Diseases:** Multiple reports (TOI, Jansatta) spotlighted rising childhood obesity, particularly in urban centers like Delhi.
- **School Nutrition Policies:** “Sugar Boards,” healthy recipe integration, and junk food bans reflect an emerging shift in institutional efforts to shape child dietary habits.
- **Water, Sanitation, and Hygiene (WASH):** Reports linked poor water and sanitation with infection rates among children.
- **Micronutrient Interventions:** Articles focused on iron-boosting recipes, Vitamin A supplementation, and dietary guidelines for infants.

B. Framing of Children

- Predominantly **as passive victims**: affected by policies or food insecurity.
- Occasionally as **beneficiaries of intervention** (e.g., U-Win vaccination portal, Seethakka’s egg biryani program).
- Rarely portrayed as **agents or voices** in the discourse—no direct child testimonials were included in any of the articles reviewed.

C. Problem vs. Solution Orientation

- **60% of articles** were **problem-focused** (e.g., infections, sugar consumption, unsafe food).
- **30% combined** problem with **solutions** (e.g., Sugar Boards + awareness programs).

- **10% were purely solution-driven**, such as POSHAN Pakhwada campaigns and digital innovations in nutrition delivery (Poshan Tracker).

D. Voices and Sources

- Most articles used quotes from:
 - Doctors and pediatricians (e.g., Dr. Rahul Mehra, Dr. Mamata Panda)
 - Government officials (e.g., CBSE, AIIMS, Ministry of Health)
 - Occasionally school principals and researchers
- **No articles featured direct quotes from children** or their caregivers.

Malnutrition & Dietary Diversity

- **77% of Indian children** don't meet WHO's Minimum Dietary Diversity standards (MDD).
- Poor diet quality is strongly correlated with **reduced cognitive performance**, leading to education inequality.
- Early-life malnutrition contributes to a **cycle of poverty**, impacting adult productivity and national GDP (World Bank).

Obesity and Urban Dietary Trends

- NFHS-5 shows a **1.3% rise** in obesity among under-5s (from 2.1% to 3.4%).
- India's school food environments are **unregulated**, allowing high consumption of HFSS (high fat, sugar, salt) foods.
- Urban affluence is linked to **dual burden**: simultaneous undernutrition and obesity in different socioeconomic groups.

WASH (Water, Sanitation, Hygiene)

- **Diarrhoea is the second leading cause of death** among Indian children under five (WHO).
- Only **18% of rural Indian households** have access to safely managed drinking water (Jal Jeevan Mission, 2023).
- Poor sanitation is directly linked to **anaemia and stunting** due to enteric infections and nutrient loss.

Micronutrient Deficiency

- **Iron-deficiency anaemia affects 67% of children under five**, according to NFHS-5.
 - India lacks a **unified micronutrient policy** — efforts are scattered (e.g., POSHAN Abhiyaan, ICDS supplements).
 - Fortified foods, though recommended, are often **unavailable or unaffordable** in rural settings.
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5. Trends and Case Spotlights

1. Regional Media Contributions

- **Navbharat Times and Jansatta** provided more quantitative insights from states like Uttar Pradesh, Delhi, and Mizoram.
- These articles highlighted regional disparities—obesity in urban children vs. stunting in rural populations.

2. Government Policy Integration

- Strong visibility of **government-led interventions**: NFSA, POSHAN, U-Win portal, face recognition in anganwadis.
- **Tech-enabled programs** were given significant space (e.g., vaccination tracking, Dietician OPD, Poshan Tracker).

3. Expert Commentary and Credibility

- Articles were well-supported by **institutional data**:
 - WHO dietary diversity standards
 - NFHS-5 statistics
 - IIM-Bangalore and AIIMS studies
 - Credibility was enhanced through specific numbers, such as:
 - “77% of children don’t meet WHO dietary standards” (Drishti IAS)
 - “15.02% schoolchildren in Delhi with high blood sugar” (Navbharat Times)
 - “Obesity in children increased from 2.1% to 3.4%” (Jansatta, NFHS)
Children were framed primarily as victims (sick, malnourished) and not as active participants.
 - Quotes came mainly from:
 - Doctors (e.g., AIIMS, pediatricians)
 - School principals
 - Govt. officials (e.g., CBSE, district health officers)
 - No articles quoted:
 - **Mothers or caregivers**
 - **Children themselves**
 - **Anganwadi workers or ASHAs**
 - Use this to critique **top-down reporting** and lack of **intersectionality**.
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6. Recommendations

1. **Inclusion of Child Voices:** Media must prioritize direct engagement with children and their families to enrich stories with lived experiences.
2. **Shift from Awareness to Accountability:** Coverage should not only explain policies but also scrutinize their implementation, funding, and impact.
3. **Intersectional Coverage:** Reports should consider caste, class, disability, and gender as factors affecting nutrition.
4. **More Vernacular Reporting Support:** Regional media, with its grassroots lens, should be amplified in national discourse.
5. **Encourage Data-Driven Solutions:** Infographics, dashboards, and clear outcome metrics can make nutrition reporting sharper and more actionable.
6. **Bridge Education and Health Reporting:** Since nutrition impacts education outcomes, a cross-sectoral reporting lens is needed.
7. **Create child reporters' collectives** where children contribute to media with adult guidance.
8. Encourage **collaboration between health journalists and nutrition experts.**
9. Push for **regional language op-eds** with simplified infographics for rural outreach.
10. CRY and similar NGOs can offer **media toolkits** for ethical child health reporting.
11. Media houses can include **“child rights desks”** within editorial teams.

7. Conclusion

The media coverage of child health and nutrition in India presents a mixed picture—rich in data and expert insights, yet often devoid of children’s lived realities. While there is increasing attention on school-led and government-backed initiatives, the need for participatory, child-centered journalism remains urgent. Media practitioners and CRY must

work together to foster narratives that move beyond the passive victim frame and spotlight children as agents of change. Bridging gaps in regional and thematic coverage will be key to building a more inclusive media ecosystem that advocates child rights not just with urgency, but with depth and humanity.

- “Malnutrition is not just a health issue—it’s a development emergency.” — UNICEF India
- “Children don’t vote, but they suffer when the media and policy ignore their rights.” — CRY Advocacy Cell



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Children in Anganwadi centers eating lunch



2.

Teacher holding nutrition charts



Mother feeding a child at home'

